

The Neuron Clinic • 450 4th Avenue #215, CHULA VISTA CA 91910-4428

HANDROP, Steve S (Legal name: Ronald Handrop) | (id #51986, dob: 12/07/1954)**Encounter Date: 11/22/2023**

Patient

Name HANDROP, RONALD (68yo, M) ID# 51986 **Appt. Date/Time** 11/22/2023 02:45PM**DOB** 12/07/1954 **Service Dept.** San Marcos**Provider** KLARI OLENSKI, DO**Insurance** Med Primary: MEDICARE-CA SOUTHERN (MEDICARE)
Insurance # : 6F02P36UG84
Med Secondary: TRICARE FOR LIFE (TRICARE - MEDICARE SUPPLEMENT)
Insurance # : 00263973200
Prescription: check now

Chief Complaint

Head Trauma

Patient states head trauma 10/18/2023

Patient's Care Team

Referring Provider: KEVIN MALONE MD: 1415 RIDGEBACK RD STE 4, CHULA VISTA, CA 91910, Ph (619) 421-4257, Fax (619) 421-6913 NPI: 1053389361

Patient's Pharmacies

RITE AID #05613 (ERX): 4348 BONITA ROAD, BONITA, CA 91902, Ph (619) 267-1617, Fax (619) 267-1137

Vitals

Ht: 5 ft 11 in (180.34 cm) 11/22/2023 02:37 pm**Wt:** 191 lbs (86.64 kg) 11/22/2023 02:37 pm**BMI:** 26.6 11/22/2023 02:37 pm**BP:** 143/96 11/22/2023 02:41 pm**Pulse:** 79 bpm 11/22/2023 02:41 pm

Allergies

Reviewed Allergies

DICLOFENAC SODIUM: Other (Moderate to severe)

Medications

Reviewed Medications

gabapentin 300 mg tablet 11/02/23 started
300 mg 3 times a day., start 11/02/2023**Jardiance 10 mg tablet** 11/22/23 entered
Take 1 tablet(s) every day by oral route.**ZyrTEC 10 mg capsule** 11/22/23 entered
Take 1 capsule(s) every day by oral route.

Vaccines

None recorded.

Problems

Reviewed Problems

- Posttraumatic headache - Onset: 11/27/2023
- Bilateral tinnitus - Onset: 11/27/2023
- Neck pain - Onset: 11/27/2023
- Closed injury of head - Onset: 11/27/2023
- Irritability and anger - Onset: 11/27/2023

Family History

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Reviewed Family History

Social History

Reviewed Social History

Substance Use

Do you or have you ever smoked tobacco?: Never smoker

Do you or have you ever used any other forms of tobacco or nicotine?: No

What was the date of your most recent tobacco screening?: 11/22/2023

Has tobacco cessation counseling been provided?: No

the Neuron Clinic Social History

1. Tobacco Use: Never Smoker

2. Alcohol: Do not Drink

3. Drug Abuse: Never User

4. Marijuana: Never User

5. Marital Status: Married

6. Occupation: Retired

Surgical History

Reviewed Surgical History

• Other

Past Medical History

Reviewed Past Medical History

Screening

Name	Score	Notes
GAD-7	3	Screening form completed by patient using Phreesia.

HPI

Patient is joined by his wife for this clinical visit who corroborated history. PCP referral with clinic note 10/18/23 reviewed and summarized; history of MVC 10/18/23. Presented to ER. Denies LOC. Had headache and neck pain. CTH negative. He fractured his elbow. This is no indication as to reason for Neurology referral.

Ronald Handrop is a 68 year old right-handed male who presents to The Neuron Clinic after a motor vehicle crash with concerns for neck pain and tinnitus. On October 18, 2023 patient was involved in a motor vehicle crash. There was no witnessed loss of consciousness or subjective amnesia. He recalls hitting the back of his head against the headrest. He has followed up with neurology, Dr. Blumenfeld, and neurosurgery, Dr. Ghosh. Recommendations include Botox for headaches and pain management for neck pain. An MRI of the brain and cervical spine were ordered, and they were hoping to review results with me today.

Currently, he states his headaches have resolved as of 1.5 weeks ago. The neck pain is described as a soreness at the sides of the neck. There is no weakness or numbness of the arms or legs, though he was having shooting pains down his 4th and 5th digit of bilateral hands. He continues to wear a soft collar and has difficulty falling asleep due to pain. He denies dizziness. He denies changes in vision. There is ringing in both ears. There is no hearing loss. Cognitive concerns include trouble with short-term memory and concentration. Mood has been irritable.

Overall, he feels that the symptoms are getting better since the injury. There has been 1 previous concussion in middle school. Loss of consciousness was associated with 1 of them, went to the ER for 1 of them, and was hospitalized for 0 of them.

Previous workup-

10/18/23 CTH (Sharp Memorial): FINDINGS: Parenchyma/Extra-axial Spaces: No CT evidence of a large territorial infarction. No hemorrhage. Nonspecific prominent extra-axial space over the right parietal convexity could be secondary to asymmetric volume loss or an underlying arachnoid cyst. No midline shift or intracranial herniation. No cerebellar tonsillar ectopia. IMPRESSION: No acute traumatic intracranial abnormality.

10/18/23: CT c-spine (Sharp Memorial): IMPRESSION:

No acute fracture or traumatic malalignment on CT of the cervical spine without contrast.

Indeterminate sclerotic lesion abutting the superior endplate on the right aspect of the T1 vertebral body measuring approximately 1 cm. Differential diagnosis would include a bone island however, malignancy/metastasis is not definitely excluded. Correlate for any history of malignancy.

Pt brought in disc of MRI brain. Appears to have a right para midline vertex arachnoid cyst (most likely) vs normal congenital variant vs asymmetric volume loss and scattered white matter T2/FLAIR hyperintensities. Additionally with what appears to have some more focused gliosis versus white matter disease of left anterior temporal lobe. Unfortunately disc is very slow to load and often skips through multiple millimeters of imaging so hard to interpret. Will await final report.

ROS**ROS as noted in the HPI****Physical Exam**

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GENERAL: Nontoxic, no acute distress, alert

HEENT: Normocephalic, atraumatic, no scleral icterus or conjunctival injection

NECK: Decreased active range of motion and discomfort in all directions, worse in flexion/extension

PSYCH: Mood and affect normal

NEURO EXAM:

Mental status: Alert. Oriented to person, place, time and situation. Appropriately interactive.

Language: Clear and fluent

Cranial Nerves:

II: VFFTC (visual fields full to confrontation)

III, IV, VI: EOMI, no ptosis. No symptoms with horizontal saccades. No symptoms with vertical saccades.

VOR is normal. VOR suppression is normal

NPC is less than 6cm

V: Sensation intact to light touch x 3 bilaterally

VII: Face symmetric without weakness

VIII: Can hear finger rub bilaterally and equally

IX, X: Voice normal, elevates palate symmetrically

XI: SCM/trapezius 5/5 bilaterally

XII: Tongue protrudes midline without atrophy or fasciculations

MSK: No involuntary movements noted. Normal Bulk and tone.

Motor:

Upper Extremity Motor:

Deltoid: 5-/5 Right, 5/5 Left

Biceps: 5/5 Right, 5/5 Left

Triceps: 5/5 Right, 5/5 Left

Grip, interossei: 5/5 Right, 5/5 Left

Lower Extremity Motor:

Hip flexion: 5/5 Right, 5/5 Left

Knee flexion: 5/5 Right, 5/5 Left

Knee extension: 5/5 Right, 5/5 Left

Plantar flexion: 5/5 Right, 5/5 Left

Dorsi flexion: 5/5 Right, 5/5 Left

Sensation:

Intact light touch in all extremities

Reflexes:

Biceps: 3+ Right, 3+ Left

Patellar: 3+ Right, 3+ Left

Coordination and Gait:

Finger-nose-finger: normal

Posture, stance, station, and arm swing normal.

Assessment / Plan

Ronald Handrop is a 68 year old right-handed male who presents to The Neuron Clinic after a motor vehicle crash on 10/18/23 with concerns for neck pain and tinnitus. He has had an MRI of the brain, though results are still pending. Will review. Follow up in 2-3 months. He is to call sooner for worsening symptoms or for any other questions/concerns.

1. Closed injury of head -

- Initial CTH without acute intracranial findings other than possible arachnoid cyst versus asymmetric volume loss. MRI brain results pending. Will request report.

S09.90XD: Unspecified injury of head, subsequent encounter

• MEDICAL RECORD REQUEST***2. Posttraumatic headache -**

- Resolved. PCP has him on gabapentin. Continue to monitor symptoms if he weans off medication since this may have inadvertently been also helping the headaches.

G44.309: Post-traumatic headache, unspecified, not intractable

3. Bilateral tinnitus -

- Improving, continue to monitor. PCP has him on gabapentin. Monitor symptoms if he weans off medication since this may have inadvertently been also helping the tinnitus. Consider ENT/Audiology evaluation if does not continue to improve.

H93.13: Tinnitus, bilateral

4. Neck pain -

- Secondarily affecting his ability to sleep

- Follow up with Neurosurgery and Pain Management

M54.2: Cervicalgia

5. Irritability and anger -

- Suspect will continue to improve as above symptoms improve. Will continue to monitor.

R45.4: Irritability and anger

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6. Mental health screening assessment -

- No symptoms on PHQ/GAD other than irritability. Continue to monitor.

Z13.30: Encounter for screening examination for mental health and behavioral disorders, unspecified

Return to Office

- to see KLARI OLENSKI, DO at San Marcos on or around 02/27/2024

Encounter Sign-Off

Encounter signed-off by KLARI OLENSKI, DO, 11/27/2023.

Encounter performed and documented by KLARI OLENSKI, DO

Encounter reviewed & signed by KLARI OLENSKI, DO on 11/27/2023 at 4:35pm

Ronald S Handrop

DOB: 12/07/1954

Age: 68 years

Gender: Male

Generalized Anxiety Disorder

Date of Visit: 11/22/2023 2:45 PM

Ronald S Handrop

Generalized Anxiety Disorder - 7 (GAD-7): Results

GAD-7 Score (Range: 0-21) *

8

The total score determines the anxiety severity based on the following scale: Minimal Anxiety (0-4); Mild Anxiety (5-9); Moderate Anxiety (10-14); Severe Anxiety (15-21)

These results do not indicate an exclusive course of treatment or serve as a standard of medical care. Variations, taking into account individual circumstances, may be appropriate.

GAD-7

Over the last 2 weeks, how often have you been bothered by the following problems?

	Not at all	Several days	More than half the days	Nearly every day
Feeling nervous, anxious or on edge	☒	☐	☐	☐
Not being able to stop or control worrying	☒	☐	☐	☐
Worrying too much about different things	☒	☐	☐	☐
Trouble relaxing	☒	☐	☐	☐
Being so restless that it is hard to sit still	☒	☐	☐	☐
Becoming easily annoyed or irritable	☐	☐	☐	☒
Feeling afraid as if something awful might happen	☒	☐	☐	☐

How difficult have these problems made it for you to do your work, take care of things at home, or get along with other people? *

Not difficult at all

GAD-7: Scoring Rules

Each response is scored as follows: Not at all = 0; Several days = 1; More than half the days = 2; Nearly every day = 3

Total score is determined by adding together the scores of all of the questions.

GAD-7: Additional information

Source: Spitzer RL, Kroenke K, Williams JBW, Lowe B. A brief measure for assessing generalized anxiety disorder. Arch Intern Med. 2006; 166: 1092-1097.